

MEDICAL FAX

FROM: [REDACTED]
TO: (800) 540-2406
SUBJECT: re: case [REDACTED]
[REDACTED]

DATE: Tue, 11/28/23, 9:53 AM EST

PAGES: 4 (not including coversheet)

INSTRUCTIONS: Hello,

I have attached [REDACTED] most recent note regarding request for MRI.

Thank you,
[REDACTED]

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[REDACTED]
[REDACTED]
This fax expires on Dec 3, 2023 9:53:43 AM. It will not be available online after that time.

Serenity Health Care, PLLC
1610 Argillite Road
Flatwoods, KY 41139
Phone (888) 756-4224 Fax (888) 258-5785

Sep 29, 2023 Fri 06:57 PM

CC Acute care visit, recently worsened back pain

HPI Painting a ceiling with a heavy paint sprayer and she leaned to the right side and her back started hurting 1 week ago. No pop, snap or tearing sensation. Pain is in the low back / upper gluteal bilateral, worse on left if she is lying down. It is radiating to upper thigh. Very painful with sitting. Pain is very similar to her prior back injury. That pain hadn't fully resolved but it had improved and was pretty manageable

She is going to class for ceramics and it's a very physical class, after working in class for a few hours her back pain becomes very difficult to manage, sitting is difficult, standing gives some relief but after time that too becomes difficult

With prior injury (4/2022) went to urgent care - given steroids, went to 8 months of PT, that did not improve pain. In the past steroids didn't help much.

She took some ibuprofen, devil's claw, willow bark.

The pain is so severe that her mood is deteriorating and it is difficult to concentrate

ROS GEN: Patient denies any fever, chills, or malaise. Feeling generally well other than current pain

PMH ADHD
h/o abuse as a child
herniated disc 2022
chronic EBV

Surgery
Tonsillectomy

Hospital stays
Age 14, serotonin shock syndrome, was on 3 SSRIs and a mood stabilizer after mis-diagnosis of bipolar
1 week stay with initial EBV

Mirena placed 2016

SH [Tobacco: Never smoker]
drinks 2 glasses of wine 5 times a week

FH ===== Structured Family History =====
No family hx breast cancer, colon cancer
GM DM, heart disease, possibly lupus
GF multiple myeloma
Mother: Meniere disease, Hashimoto thyroiditis, Bipolar disorder
Father: Substance abuse

Allergies penicillin

Printed By 11/28/2023 9:51:33 AM

AmazingCharts

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Sep 29, 2023 Fri 06:57 PM

Meds

1) Slynid 4 mg oral tablet, 1 po qday

PE

Telehealth visit

Appearance: WN, WD, NAD, patient appears uncomfortable but is without rapid respiratory rate. Pt appears alert

Eyes: PERRL, EOM appear intact. No lacrimation evident, pupils appear normal size for room light

OP: no rhinorrhea present, no yawning during visit

Skin: no jaundice, rash, piloerection or bruising visible. No visible skin lesions

Neurologic: non-focal exam via video monitor, pt is not restless on exam. No tremor is noted

Musculoskeletal: normal station. Standing and pacing, clearly in discomfort

Psychiatric: alert and oriented to person, place, time; appropriate affect, tearful at time when discussing her pain, no psychomotor agitation evident, insight and judgement intact; remote and recent memory intact, no suicidal or homicidal ideation evident, no hallucinations or delusions evident, conversation WNL, no tangential or circumferential qualities noted. Thought content and processes appear to be WNL for this patient

A/P

Lumbar discogenic pain (M51.26): pain has resumed and is severe. Pt has completed PT for 8 months with no long term improvement. Try prednisone and toradol now to improve function, if no better pt will call. Get MRI to evaluate severity of likely disc herniation. Pt would prefer to avoid surgery if possible. D/w pt options. Gave pt resources to try some stretches at home if tolerated. Counseled more than 1/2 visit, total time = 30 minutes. Pt will call if she needs a statement to reduce class hours due to pain. Try tens unit.

Chronic back pain (R52): pain was bearable until recently. Has not had imaging. All pain started with acute injury 4/22

PRESCRIBE: predniSONE 20 mg oral tablet, 1 po qday with food for 5 days, # 5, RF: 0.

(Transmitted by Kathie Watson-Gray, MD)

PRESCRIBE: ketorolac 10 mg oral tablet, 1 po qday with food for 3 days, # 3, RF: 0.

(Transmitted by Kathie Watson-Gray, MD)

marshall pharmacy

RTO 5 weeks, call before then if pain worsens. If pain develops into numbness or weakness of the leg, pt will go to Baptist hospital ER for evaluation immediately

PROVIDED: Patient Education (9/29/2023) Viniyoga for low back pain, do not do exercises that cause or increase pain

SMMC

ORDERED/ADVISED: Order Date 09-29-2023

- MRI LS spine (R52, M51.26)

- z please fax results to 888-258-5785 (R52, M51.26)

This service was provided via telemedicine with real-time interactive video and audio communication. This visit was conducted with the patient in their home in Ohio. Pt received and agreed with telehealth consent and requested a video visit today.

11/28/2023 9:51:33 AM

Amazon Chorus

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From [REDACTED]

18882585785

11/28/2023 08:54:03 PST

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[REDACTED] Sep 29, 2023 Fri 06:57 PM

Electronically Signed By: [REDACTED]

9/29/23 8:28 PM

Printed By: [REDACTED] 11/28/2023 9:51:33 AM

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